Carry your treatment details

Which treatment this guide covers This guide covers NCCP 00353 osimertinib on its own: T790M-positive advanced disease, first-line activating-EGFR advanced disease, or completely resected stage IB–IIIA Ex19del/L858R adjuvant treatment. The newer licensed post-chemoradiation setting and osimertinib with platinum/pemetrexed need their own applicable plan; their study results and support medicines are not added to this monotherapy suite.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Osimertinib monotherapy

NCCP 00353 v5

The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Contact the oncology team urgently for new or worsening breathlessness, cough or fever; fainting, palpitations or chest pain; blistering/peeling skin or mouth/eye/genital sores; unusual bruising, bleeding or marked pallor; severe diarrhoea, dehydration or inability to drink. New eye pain, light sensitivity or blurred vision needs prompt advice. Severe breathing difficulty, collapse or swelling of the tongue/ throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

https://sactcheck.com/patient/00353/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending